TE-05 Hypothermia / Frostbite

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 30, 2026

Protect from further heat loss and handle gently. Severe hypothermia can mimic death. Full resuscitation and rewarming occur together unless there is obvious lethal injury or the patient is frozen solid.

Recognition and immediate field flow

Step	Approved action
1. Shelter and insulate	Move to warm shelter, remove wet clothing, dry the patient, insulate all exposed surfaces, and obtain temperature when feasible without delaying treatment.
2. Localized or systemic?	Frostbite: do not rub. If refreezing can be prevented, rewarm in 37-40 C (99-104 F) water for 20-30 minutes. Systemic hypothermia: support airway, glucose, warming, and ECG.
3. Pulse check	For severe hypothermia, assess for signs of life and pulse for at least 60 seconds. If absent, begin the age-appropriate cardiac-arrest pathway with hypothermia modifications and concurrent rewarming.
4. Core warming	Use warm blankets and protected heat packs to axillae and groin. Keep chemical heat away from frostbitten tissue. Use warmed fluids when available.
5. Perfusion	For hypotension or poor perfusion, use warmed isotonic crystalloid in reassessed aliquots: adult 500 mL; pediatric 20 mL/kg. Do not use a routine fixed maximum.
6. Hypothermic arrest	Below 30 C (86 F): full resuscitation with rewarming. For VF/pVT, defibrillate once. If unsuccessful, defer further shocks and epinephrine until 30 C (86 F) or greater; contact Medical Control early.

Provider-level actions

Level	Approved actions
EMT	Remove wet clothing; dry and insulate; apply protected heat packs to axillae and groin; check glucose; provide gentle handling; and reassess temperature, airway, breathing, perfusion, and mental status.
AEMT	All EMT care plus IV/IO access and warmed isotonic crystalloid when available for hypotension or poor perfusion, with reassessment after each aliquot.
Paramedic	All prior care plus cardiac monitoring and the hypothermic-arrest sequence below 30 C (86 F): full resuscitation with rewarming; one VF/pVT defibrillation attempt; then defer further shocks and epinephrine until 30 C or greater.

Frostbite safeguards

- Do not rub or massage frostbitten tissue, debride blisters, or apply direct chemical heat to frostbitten tissue.
- Do not rewarm frostbite if refreezing is possible. When a sustained thaw can be assured, use 37-40 C water for 20-30 minutes.
- Protect rewarmed tissue from refreezing and arrange prompt definitive evaluation.

TE-05 | HYPOTHERMIC ARREST, TRANSPORT, AND HANDOFF

Hypothermic cardiac arrest

Topic	Required action
Below 30 C	Provide full resuscitation and concurrent rewarming. For VF/pVT, deliver one shock. If unsuccessful, defer further shocks and epinephrine until core temperature is 30 C (86 F) or greater.
Termination	Do not use a fixed temperature alone to stop resuscitation. Continue unless there is obvious lethal injury or the patient is frozen solid; obtain early Medical Control input.
ETCO2	Do not use ETCO2 alone to predict outcome in hypothermic cardiac arrest.
Destination	Contact Medical Control early and consider an ECLS-capable destination when feasible without compromising timely resuscitation and transport.

Transport and documentation

Topic	Required action
Transport	Prevent additional heat loss, continue gentle handling and protected warming, reassess ECG, glucose, perfusion, temperature, and respiratory status, and give early destination notification for severe exposure or arrest.
Handoff	Report estimated exposure, wet-clothing removal, temperature trend, mental status, ECG/rhythm, pulse-check findings, rewarming methods, fluids, defibrillation/medication modifications, and response to care.
Linked care	Use the applicable airway, shock, seizure, cardiac-arrest, and trauma pathway. For cold-water submersion, also use TE-03 Drowning / Submersion Injury.

Critical reminder: In severe environmental hypothermia, apparent death can be deceptive. Gentle handling, full resuscitation with rewarming, early Medical Control consultation, and appropriate destination planning are central safeguards.

Release note

- This direct source PDF replaces the prior imported sheet and matches Claiborne-approved TE-05 app content reconciled October 30, 2026.